

Blood Transfusion Administration and Reaction Management

Policy Number	POL-LAB-006	Owner / Department	Laboratory / Blood Bank (Transfusion Service) · Nursing
Effective Date	2023-06-01	Version	3.2
Last Revised	2026-03-01	Next Review	2028-03-01
Approved By	Patient Safety & Quality Committee	Applies To	All staff who order, collect for, or administer blood components

1. Purpose

To ensure safe administration of blood components and a prompt, standardized response to suspected transfusion reactions, consistent with AABB Standards and FDA requirements.

2. Policy — Pre-Transfusion Verification

- Two qualified staff perform an independent bedside check matching the patient's two identifiers, the blood bank record, and the unit label/component before starting.
- Obtain baseline vital signs immediately before transfusion; remain with the patient for the first 15 minutes, the period of highest risk.

3. Suspected Transfusion Reaction — Immediate Actions

IMPORTANT — Stop the transfusion immediately at the first sign of a reaction (fever, chills, hypotension, dyspnea, back/flank pain, hemoglobinuria, or sudden deterioration). Keep the IV line open with normal saline using new tubing, and notify the ordering provider and the Blood Bank at once. An acute hemolytic reaction is a SEV-1 event.

- Stop the transfusion; disconnect the unit but keep the IV line patent with 0.9% normal saline via new tubing.
- Recheck the patient's identifiers against the unit label and paperwork at the bedside.
- Assess and monitor vital signs; treat symptoms as ordered.
- Notify the ordering provider and the Blood Bank immediately.
- Return the blood unit and attached administration set to the Blood Bank; collect post-reaction blood and urine specimens per protocol.
- Complete the transfusion reaction report form and file an event report per POL-RM-013.

4. Reporting Requirements

- The Blood Bank investigates every suspected reaction and notifies the component supplier(s) of implicated units.
- Any transfusion- or collection-related fatality is reported by the Blood Bank Medical Director to the FDA Center for Biologics Evaluation and Research (CBER) as soon as possible, with a written report within 7 days of the fatality.

- Participation in the CDC NHSN Hemovigilance Module is maintained for trended surveillance.

5. Documentation Requirements

- Component type and unit number; start/stop times and volume infused.
- Symptoms, onset time, vital-sign trend, and treatment.
- Bedside re-verification result; specimens collected and returned to the Blood Bank.
- Provider and Blood Bank notification times.

6. References

- AABB, Standards for Blood Banks and Transfusion Services.
- FDA, 21 CFR 606.170 — fatality reporting to CBER.
- Related: POL-PS-008 (patient identification), POL-RM-013 (severity & reporting).